BASELINE → TREATMENT → FOLLOW-UP

# When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. National Cancer Control Programme (NCCP) remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

| Check | Before first dose | Early visits | Week 6 · around C3 | Week 12 · around C5 | Later / after stopping |
|---|---|---|---|---|---|
| **Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)** | Baseline before starting | Earlier if symptoms or cardiac concern | Not routinely repeated just because week 6 arrives | Repeat at 12 weeks; earlier checks may be needed | Every 12 weeks during treatment; after stopping, the EMA cardiac follow-up schedule is every 6 months until 24 months. Individual cardiac risk may require more. |
| **Blood tests** | Baseline full blood count (FBC), renal and liver profile | Additional tests if needed | Repeat before this cycle | Repeat before this cycle | Every 6 weeks |
| **electrocardiogram (ECG) / rhythm** | Baseline ECG in EMA cardiac assessment; it does not replace echo. | Symptoms or clinical indication | Clinical plan | Clinical plan | Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring |
| **Cancer scans / markers** | Document the disease sites and imaging purpose | Assess symptoms at visits | Scan only if due / indicated | Metastatic: often near a response review; exact date agreed | Response scans commonly every 2–4 months; individualise. Markers supplement clinical review and imaging. |
| **dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health** | If endocrine / ovarian suppression is planned: assess need | Not a routine trastuzumab test alone | Timing depends on risk / result | Separate from the echo schedule | DEXA and repeat timing depend on endocrine treatment, fracture risk and results. |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00272 v4 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · retrieved 25 October 2026

THE WHOLE TREATMENT JOURNEY

# Trastuzumab subcutaneous (SC) · metastatic: treatment and checks

**HER2-positive metastatic breast cancer · SC component**

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before treatment | First cycles | Around 12 weeks | Continuing treatment | Completion / change |
|---|---|---|---|---|---|
| **Cancer medicine** NCCP 00272 | Confirm setting, prior doses and formulation | SC trastuzumab every 3 weeks | Review benefit, symptoms and heart function | Continue while beneficial and tolerated | Review benefit, toxicity and goals; continue or change treatment without a fixed one-year end date. |
| **Surgery** METASTATIC | Local procedures only if indicated | No automatic curative surgical sequence | Coordinate procedures with oncology | Symptom / site-specific decisions | Local follow-up as indicated |
| **Radiotherapy** EARLY · METASTATIC | Assess indication and planned site | Timing depends on the clinical need | Agree any hold / continuation with radiation oncology | radiotherapy (RT) is site / symptom directed. Agree any treatment hold with both teams. | Radiation effects and surveillance follow the local plan |
| **Blood tests** NCCP 00272 | Baseline FBC, renal, liver | Every 6 weeks | Week 12 is another six-week blood-check point for trastuzumab alone; companion therapy may require more frequent tests. | Every 6 weeks | Follow abnormalities and the next treatment plan |
| **Heart checks** NCCP 00272 · SMPC | Baseline echo / MUGA; cardiac assessment | Symptoms can trigger earlier checks | Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles | Repeat every 12 weeks; 4–8 weeks may be needed with impairment | Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail. |
| **Scans / markers** EARLY · METASTATIC · MARKERS | Record baseline metastatic sites and scan purpose | Review symptoms at every visit | Response imaging according to the individual disease plan | Usually reassess about every 2–4 months; individualise | Use imaging and clinical assessment together; tumour markers are supplementary. |

| Care / check | Before treatment | First cycles | Around 12 weeks | Continuing treatment | Completion / change |
|---|---|---|---|---|---|
| **Endocrine / bone health**<br>EARLY · BONE | Check hormone receptor status and endocrine indication | Endocrine timing coordinated with the wider plan | DEXA if indicated by endocrine / fracture risk | May overlap trastuzumab when clinically appropriate | Longer endocrine and bone follow-up can outlast HER2 treatment |

# What each check is for

## 1 Blood tests

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time)

Results help determine whether treatment can proceed.

**Ask / act:** The team confirms booking, results and next dose.

Sources: NCCP 00272

## 2 Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

**Ask / act:** Keep the booked scan; report symptoms between scans.

Sources: NCCP 00272 · SMPC

## 3 Cancer imaging and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Use the result for a specific clinical decision.

**Ask / act:** Ask which scan is planned and who reviews it.

Sources: EARLY · METASTATIC · MARKERS

## 4 Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

**Ask / act:** Record booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00272

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21